

Policy on Obstetric Care

«Hospital Name»

Document control

Document No.	HCO/COP/POL/10	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Obstetric In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Obstetric care is organised and delivered safely, with high-risk cases identified and cared for by competent staff, ante-natal services following written guidance, and mothers treated with dignity and privacy throughout.

1. Purpose

This policy explains how «Hospital Name» organises and delivers safe obstetric care, identifies and manages high-risk obstetric cases, runs ante-natal services under written guidance, welcomes birth companions, treats mothers respectfully and confidentially, explains danger signs, monitors mother and baby through labour and after birth, and — where these services are within scope — provides neonatal intensive care and Assisted Reproductive Technology services in line with the law.

This policy does not cover general patient assessment, informed consent procedures, or newborn care in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all obstetric, nursing, and neonatal staff at «Hospital Name». Sections on neonatal intensive care (5.9) and Assisted Reproductive Technology (5.10) apply only if «Hospital Name» provides those specific services.

3. Policy standards

«Hospital Name» delivers obstetric care under written guidance covering pregnant-patient assessment, ante-natal, peri-natal, and post-natal care, identifies and either manages or refers high-risk obstetric cases using competent staff, runs ante-natal services with a complete ante-natal card for every patient, welcomes birth companions, treats mothers with privacy, respect, and confidentiality, explains danger signs, monitors labour and the post-natal period for key risks, and — where in scope — provides NICU care and follows Assisted Reproductive Technology law.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not deliver obstetric care without written guidance covering, at minimum, pregnant-patient assessment (nutrition, immunisation, education), ante-natal care, and peri-natal and post-natal care.
- Do not leave whether «Hospital Name» provides high-risk obstetric care undefined, or leave what counts as "high-risk" undefined.
- Do not manage a high-risk obstetric case without competent workforce and facilities for both mother and neonate — refer proactively to another centre if high-risk care is out of scope.
- Do not let anyone other than a competent doctor or nurse — qualified, experienced, or trained — care for a high-risk obstetric case.
- Do not run ante-natal services without written guidance covering assessment, immunisation, diet counselling, and frequency of visits, and without a complete ante-natal card (or equivalent) for every patient.
- Do not skip monitoring foetal heart rate during labour, the progression of labour, and post-natal monitoring for post-partum haemorrhage.
- Where «Hospital Name» provides high-risk obstetric care, do not do so without a NICU staffed with competent, qualified paediatric/neonatal personnel.
- Where «Hospital Name» provides Assisted Reproductive Technology services, do not provide them without the required infrastructure, ART lab, operation theatre, competent team, consent process, and screening/SOPs, and without complying with the Assisted Reproductive Technology (Regulation) Act as amended.

5. What we do

5.1 Organise obstetric care under written guidance

Written guidance based on standard treatment guidelines or sound clinical practice governs the organisation and delivery of obstetric care. At a minimum it covers three things: assessment of pregnant patients, including nutrition, immunisation, and education; ante-natal care guidelines; and peri-natal and post-natal care guidelines. «Hospital Name» states

in this guidance whether it provides care for high-risk obstetric cases, and defines what counts as high-risk, in line with best clinical practice. Staff are trained in managing obstetric emergencies, based on evidence-based clinical practice guidelines.

5.2 Identify and manage high-risk obstetric cases

Ante-natal examinations guide the early identification of high-risk obstetric cases, which have more frequent check-ups where appropriate. Where high-risk obstetric care isn't in «Hospital Name»'s scope, patients are proactively referred to an appropriate centre. Where «Hospital Name» does provide high-risk care, it has a competent workforce and the facilities to care for both mothers and neonates in these cases.

5.3 Use competent staff for high-risk cases

Persons caring for high-risk obstetric cases are competent — this includes both doctors and nursing staff, not doctors alone. Competency is based on qualification, experience, or training, or a combination of these.

5.4 Provide ante-natal services under written guidance

Written guidance governs ante-natal services, covering at minimum four things: assessment, immunisation, diet counselling, and frequency of visits. Every patient has an ante-natal card (or an equivalent record), complete with her medical and obstetric history.

5.5 Welcome birth companions

Staff encourage and welcome the presence of a birth companion during labour.

5.6 Treat mothers with dignity and privacy

The Labour/Delivery Room has provisions for privacy — for example, curtains, partitions between tables, or non-see-through windows. Staff treat the pregnant woman and her companion cordially and respectfully, and her confidentiality is protected throughout her stay.

5.7 Explain danger signs

A treating doctor, or a doctor from the treating team, explains danger signs and important care activities to the expecting mother and her companion.

5.8 Assess and monitor through pregnancy and birth

Maternal nutrition is part of every obstetric patient's assessment, documented in her ante-natal card. It's preferable for a dietician to carry out this assessment. Appropriate peri-natal and post-natal monitoring is performed — at a minimum, this covers foetal heart rate during labour, the progression of labour, and post-natal monitoring for post-partum haemorrhage.

5.9 Provide neonatal care for high-risk cases, where this service is offered

Where «Hospital Name» provides care for high-risk obstetric cases, it also has a NICU — level I, II, or III, depending on scope — with appropriate equipment and staff, including a

competent paediatrician or neonatologist and nurses, qualified and trained for neonatal care. This section does not apply where high-risk obstetric care is outside «Hospital Name»'s scope.

5.10 Follow Assisted Reproductive Technology law, where this service is offered

Where «Hospital Name» provides Assisted Reproductive Technology (ART) services, it maintains the required infrastructure, an ART lab, an operation theatre, a competent ART team with trained counsellors and counselling techniques, a consent process, and patient screening and SOPs following national guidelines. All of this complies with the Assisted Reproductive Technology (Regulation) Act as amended. This section does not apply where ART services are outside «Hospital Name»'s scope.

6. Stop-work authority

Do not manage a high-risk obstetric emergency without trained staff or without applying defined admission/referral criteria; do not continue labour without foetal heart rate monitoring; do not provide Assisted Reproductive Technology services in breach of the ART Act as amended.

Stop-work applies to non-emergent obstetric care decisions and ART service delivery. A mother already in active labour or an obstetric emergency already in progress continues receiving the best available qualified care while the gap is escalated.

The person who stops tells the Obstetric In-Charge and the Medical Superintendent the same shift. Refusing unsafe obstetric or ART care is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced; receives stop-work notifications the same shift.
Obstetric In-Charge	Oversees delivery of obstetric care; ensures written guidance, trained staff, and required facilities are in place; receives stop-work notifications the same shift.
Treating obstetricians and doctors	Apply high-risk identification criteria; explain danger signs to mothers and companions; document assessment and monitoring in ante-natal cards.
Obstetric and nursing staff	Follow written guidance for ante-natal, peri-natal, and post-natal care; monitor foetal heart rate during labour, labour progression, and post-natal post-partum haemorrhage; support birth companions; maintain privacy and dignity in the

	Labour/Delivery Room.
ART team (where ART services are in scope)	Follows ART Act requirements and national ART guidelines; maintains consent process, patient screening, and SOPs.
NICU staff (where high-risk obstetric care is in scope)	Provides competent paediatric/neonatal care using qualified and trained personnel; maintains appropriate equipment.

8. Quality monitoring

Monitoring area	What is monitored
Written guidance	Written guidance covering minimum three areas (pregnant-patient assessment, ante-natal care, peri-natal and post-natal care) in place and current
High-risk identification	High-risk cases identified via ante-natal examination; proactive referral where high-risk care is out of scope; "high-risk" criteria defined in written guidance
Ante-natal written guidance	Written guidance for ante-natal services covering all four minimum areas (assessment, immunisation, diet counselling, frequency of visits)
Ante-natal card	Complete ante-natal card (or equivalent) maintained for every patient, with medical and obstetric history including maternal nutrition assessment
Labour and post-natal monitoring	Foetal heart rate during labour, labour progression, and post-natal PPH monitoring performed and documented for every patient
Privacy and dignity	Privacy provisions in Labour/Delivery Room; mothers and companions treated respectfully; confidentiality maintained
Danger signs	Danger signs and important care activities explained to mother and companion by a treating doctor; documented
NICU (where in scope)	NICU with appropriate equipment and competent paediatric/neonatal staff in place where high-risk obstetric care is provided
ART compliance (where in scope)	Required ART infrastructure, lab, OT, team, consent, and screening/SOPs in line with the ART Act where ART services are provided

9. Training and staff acknowledgement

All obstetric, nursing, and neonatal staff shall be familiar with the written guidance applicable to their work, including high-risk obstetric criteria, ante-natal care requirements, labour and post-natal monitoring requirements, privacy and dignity obligations, and — where applicable — NICU care standards and ART Act requirements.

I have read the Policy on Obstetric Care of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all obstetric, nursing, and neonatal staff at «Hospital Name».

11. Abbreviations

Abbreviation	Meaning
ART	Assisted Reproductive Technology
NICU	Neonatal Intensive Care Unit
OT	Operation Theatre
PPH	Post-Partum Haemorrhage
SOP	Standard Operating Procedure

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.10.a	Commitment*	Section 5.1 addresses written guidance covering at minimum three areas: assessment of pregnant patients including nutrition, immunisation, and education; ante-natal care guidelines; and peri-natal and post-natal care guidelines.
COP.10.b	Commitment	Sections 5.1 and 5.2 address whether high-risk obstetric care is provided, the definition of high-risk, early

		identification via ante-natal examination, more frequent check-ups for high-risk cases, proactive referral where high-risk care is out of scope, and staff training in obstetric emergency management. Stop-work authority (Section 6) also applies to this element.
COP.10.c	Commitment	Section 5.3 addresses competency requirements for persons caring for high-risk obstetric cases — both doctors and nursing staff — based on qualification, experience, or training; and proactive referral when high-risk care is outside scope. Stop-work authority (Section 6) also applies to this element.
COP.10.d	Commitment*	Section 5.4 addresses written guidance for ante-natal services covering at minimum four areas: assessment, immunisation, diet counselling, and frequency of visits. Also addresses the mandatory ante-natal card (or equivalent) with complete medical and obstetric history for every patient.
COP.10.e	Achievement	Section 5.5 addresses welcoming and encouraging the presence of a birth companion during labour.
COP.10.f	Commitment	Section 5.6 addresses privacy provisions in the Labour/Delivery Room (e.g., curtains, partitions, non-see-through windows), cordial and respectful treatment of the pregnant woman and companion, and confidentiality throughout her stay.
COP.10.g	Commitment	Section 5.7 addresses

		explanation of danger signs and important care activities to the expecting mother and companion by the treating doctor or a doctor from the treating team.
COP.10.h	Commitment	Section 5.8 addresses maternal nutrition as a mandatory component of every obstetric patient's assessment, documented in the ante-natal card, with a dietician performing the assessment where possible.
COP.10.i	Commitment	Section 5.8 addresses appropriate peri-natal and post-natal monitoring, covering at minimum three items: foetal heart rate during labour, the progression of labour, and post-natal monitoring for post-partum haemorrhage. Stop-work authority (Section 6) also applies to this element.
COP.10.j	Commitment	Section 5.9 addresses NICU provision (level I, II, or III) with appropriate equipment and competent paediatric/neonatal personnel where high-risk obstetric care is in scope. Explicitly scope-conditional: does not apply where high-risk obstetric care is outside scope.
COP.10.k	Commitment	Section 5.10 addresses ART service provision including required infrastructure, ART lab, operation theatre, competent ART team with trained counsellors, consent process, patient screening, and SOPs following national guidelines, in compliance with the Assisted Reproductive Technology (Regulation) Act as

		amended. Explicitly scope-conditional: does not apply where ART services are outside scope. Stop-work authority (Section 6) also applies to this element.
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13. Required Records/Evidence Checklist

Written guidance — obstetric care

- Written guidance based on standard treatment guidelines or sound clinical practice covering pregnant-patient assessment (nutrition, immunisation, education), ante-natal care guidelines, and peri-natal and post-natal care guidelines.
- Written guidance stating whether high-risk obstetric care is provided and defining what counts as high-risk.
- Evidence of staff training in managing obstetric emergencies.

High-risk identification and management

- Records showing high-risk cases identified via ante-natal examinations.
- Evidence of more frequent check-ups for high-risk cases where appropriate.
- Referral records for high-risk cases where high-risk care is out of scope.
- Evidence of competent workforce and facilities for high-risk care where provided.

Ante-natal services

- Written guidance for ante-natal services covering assessment, immunisation, diet counselling, and frequency of visits.
- Complete ante-natal card (or equivalent) for every patient, with medical and obstetric history including maternal nutrition assessment.

Privacy, dignity, and danger signs

- Evidence of privacy provisions in the Labour/Delivery Room (e.g., curtains, partitions, non-see-through windows).
- Records of danger signs explained to mothers and companions by a treating doctor.

Labour and post-natal monitoring

- Records of foetal heart rate monitoring during labour.
- Records of labour progression monitoring.
- Records of post-natal monitoring for post-partum haemorrhage.

NICU (where high-risk obstetric care is in scope)

- Evidence of NICU provision (level I, II, or III) with appropriate equipment and competent paediatric/neonatal staff.

ART services (where in scope)

- Evidence of required ART infrastructure, ART lab, and operation theatre.
- Evidence of competent ART team including trained counsellors.
- ART consent process documentation and patient screening records.
- SOPs following national guidelines for ART services.
- Evidence of compliance with the Assisted Reproductive Technology (Regulation) Act as amended.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.10.
2. Guidebook interpretation supplied for COP.10.a through COP.10.k.
3. Assisted Reproductive Technology (Regulation) Act as amended (reference for COP.10.k ART compliance requirements).

Disclaimer

This policy reorganises the supplied COP.10 objective elements and approved plain-language content into policy format. The stop-work text is reproduced exactly as supplied. Mandatory requirements and their stated modal strength have been retained. The three minimum written-guidance areas (COP.10.a), four minimum ante-natal written-guidance areas (COP.10.d), three minimum monitoring items (COP.10.i), and six ART scope elements (infrastructure, ART lab, operation theatre, competent team, consent process, screening/SOPs) are preserved exactly as stated. COP.10.j (NICU) and COP.10.k (ART) are explicitly presented as scope-conditional, not universal requirements.